

Preliminary Real-World Evidence on the Pharmacy Benefit Uptake of Biosimilar Products for Self-Insured Employers

Elizabeth Brewster, PharmD; Catherine Berger, PharmD; Courtney Keefe, PharmD; Tu Doan, PharmD; Matthew Harman, PharmD, MPH

Background

- Biosimilar products, which closely mimic their originator biologic product, have saved the US healthcare system ~\$36 billion since the first launch in 2015.¹
- Almost 2.7 billion days of patient therapy using biosimilars have been recorded with no meaningful differences in safety or clinical outcomes.²
- Despite significant savings for patients and health plans, plus proven equivalency, adoption of biosimilars in the market has been slower than anticipated due to insurer coverage decisions, high rebates, and prescriber acceptance.
- “Product-hopping” is a strategy used by some PBMs (Pharmacy Benefit Managers), health plans and manufacturers to shift patients to newer, often higher-priced brand products instead of a biosimilar with lower net cost, which is also contributing to the slow rate of biosimilar adoption.²
- Branded adalimumab utilizers switching to either Skyrizi (risankizumab) or Rinvoq (upadacitinib) grew by 120% from 2022-2023 in the United States.³
- Only one of the “Big Three” PBMs had excluded branded adalimumab in favor of a biosimilar in the first quarter of 2024.

Objectives

- To evaluate whether members who used branded adalimumab switched to the formulary preferred biosimilar, switched to a different branded biologic product, or remained on branded adalimumab after it became a formulary exclusion.
- To determine why a member may not stay on the preferred biosimilar post-switch and instead switch back to the originator product or a different branded biologic.
- To analyze which conditions may be associated with a member switching to a different branded biologic product instead of the formulary preferred biosimilar.

Methods

- Pharmacy claims data from 284 self-insured employers implementing a branded adalimumab exclusionary formulary were generated by reporting tools from a contracted PBM.
 - A fourteen-month time frame was used and split into a pre-exclusion period (9/1/23-3/31/24) and a post-exclusion period (4/1/24-10/31/24).
- Key metrics included: Product name; Member account ID; Claim fill date; Member age; Member gender; Diagnosis code; Pharmacy name.
- Internal prior authorization review system was utilized for patient specific product switch reasoning.
- AI = autoimmune; AS = ankylosing spondylitis; CD = Crohn’s disease; HS = hidradenitis suppurativa; JIA = juvenile idiopathic arthritis; PsO = psoriasis; PsA = psoriatic arthritis; RA = rheumatoid arthritis; UC = ulcerative colitis

Limitations

- ICD-10 codes were used to decipher indications which are reliant upon prescriber accuracy, and it was assumed that each member had one only autoimmune condition for which they used biologics.
- The included biologic products do not have the exact same indications or dosing schedules, which limited the ability to track adherence.
- Data points for PsO and PsA were combined due to the number of members who have been diagnosed with both conditions.
- Natural progression of autoimmune conditions, including antibody development, was not taken into account.

Results

Table 1: Post-exclusion Switch Outcomes

Switch Type	Members
Direct switch	1940 (79%)
Indirect switch	220 (9%)
Nonswitch	95 (4%)
Step through biosimilar to other brand product	130 (5%)
Switch back	56 (2%)
Total	2,441

Definitions

Direct switch = Formulary preferred adalimumab biosimilar

Indirect switch = Other biologic branded product sharing at least one indication with adalimumab

Nonswitch = Stayed on branded adalimumab

Step through = Fill of other biologic branded product after at least one fill of formulary preferred biosimilar

Switch back = Refill of branded adalimumab after at least one fill of formulary preferred biosimilar

Table 2: Indirect Switch Product Key

Brand Name	Generic Name
Actemra	tocilizumab
Cimzia	certolizumab
Cosentyx	secukinumab
Enbrel	etanercept
Kevzara	sarilumab
Orencia	abatacept
Otezla	apremilast
Rinvoq	upadacitinib
Skyrizi	risankizumab
Stelara	ustekinumab
Taltz	ixekizumab
Tremfya	guselkumab
Velsipity	etrasimod
Xeljanz	tofacitinib
Zeposia	ozanimod

Figure 1: Indirect Switch Product Utilization

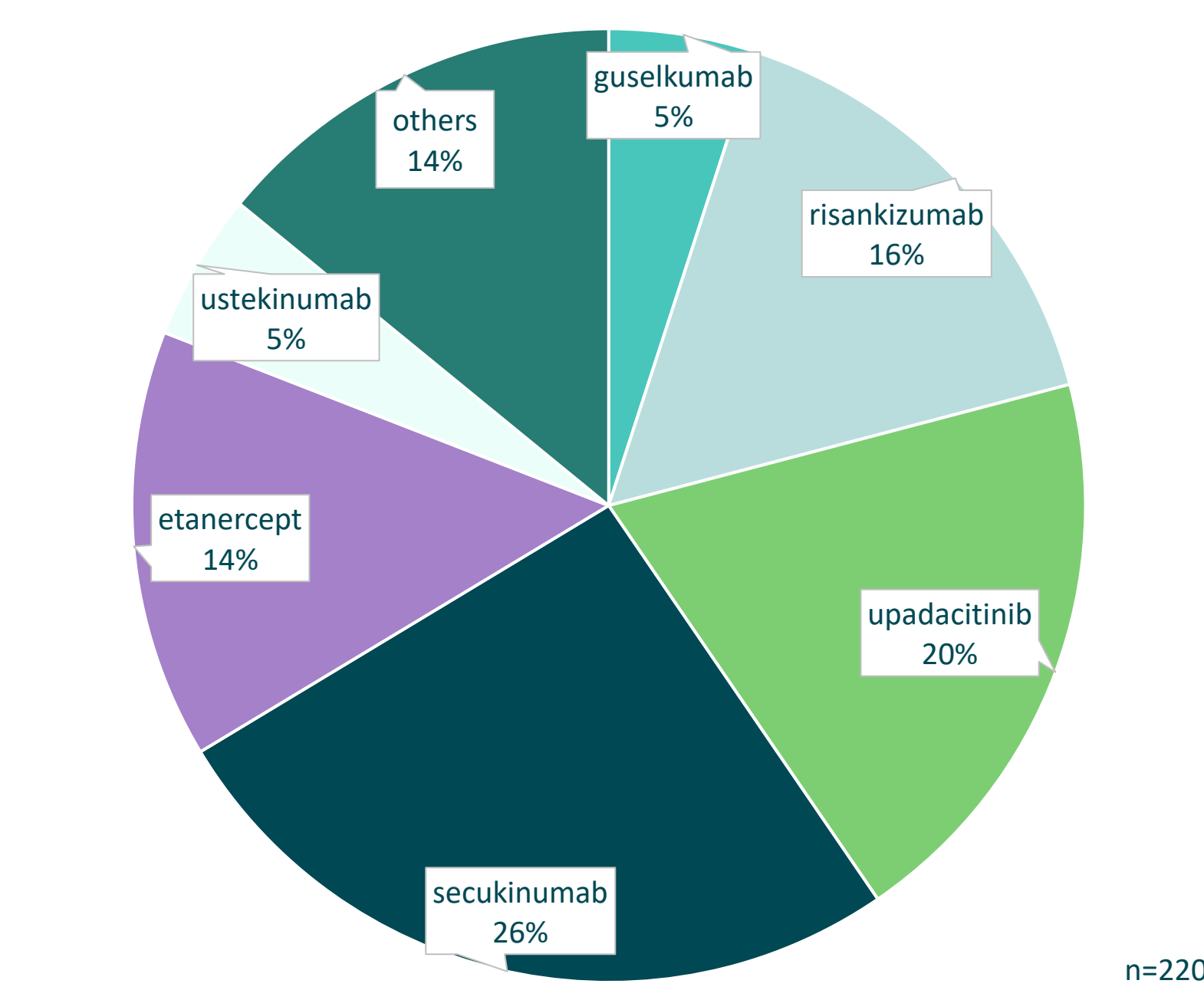


Figure 2: Disease States of Indirect Switch Members

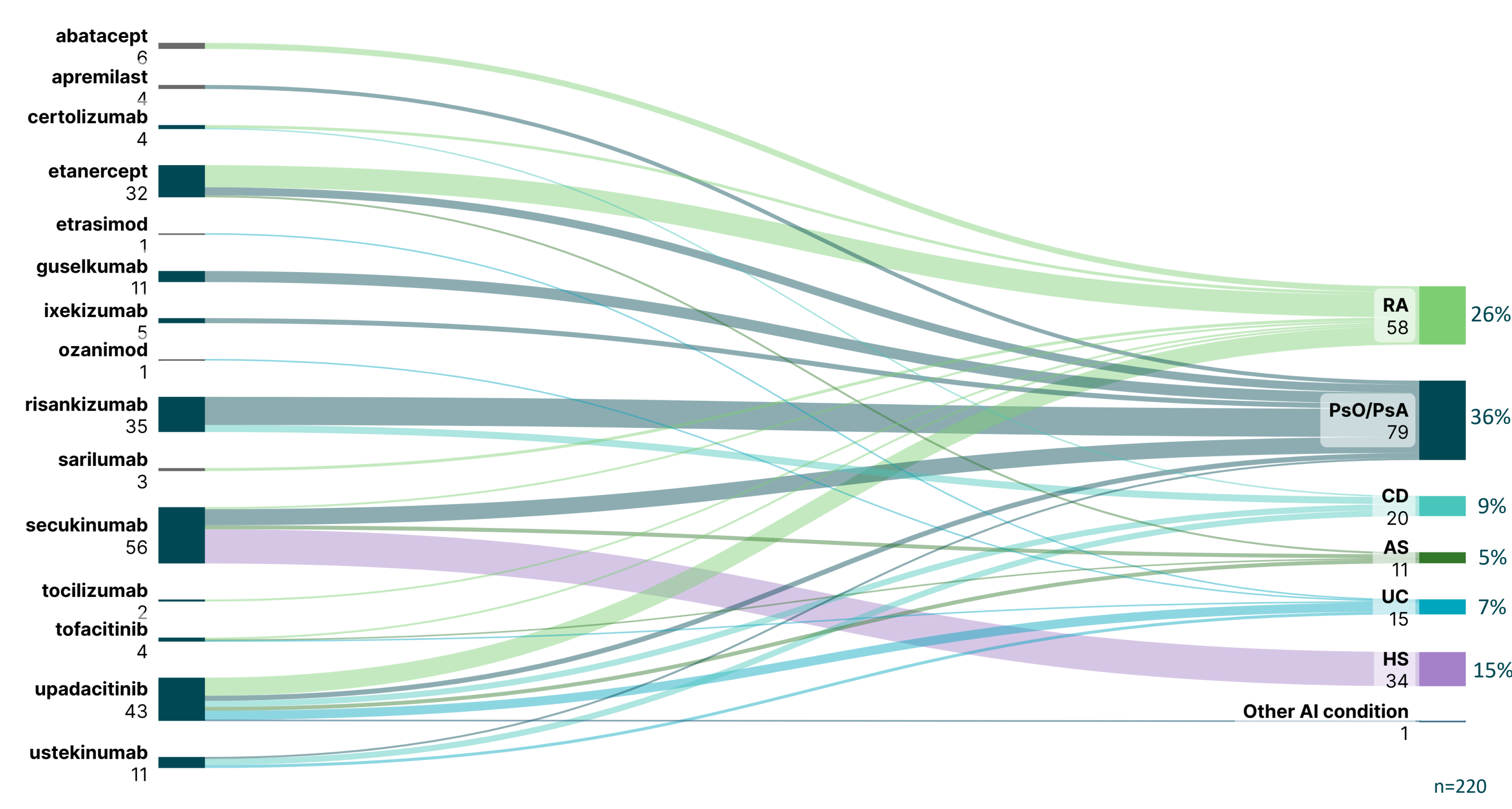


Table 3: Utilization of Indirect Switch Products per Condition

Condition	AS (n=11)	CD (n=20)	HS (n=34)	PsO/PsA (n=79)	RA (n=58)	UC (n=15)
Product with Most Utilization per Condition	secukinumab (36%) upadacitinib (36%)	risankizumab (35%)	secukinumab (100%)	risankizumab (35%)	etanercept (38%)	upadacitinib (60%)

Figure 3: Adalimumab Indications for Switch Back Members vs. BoB

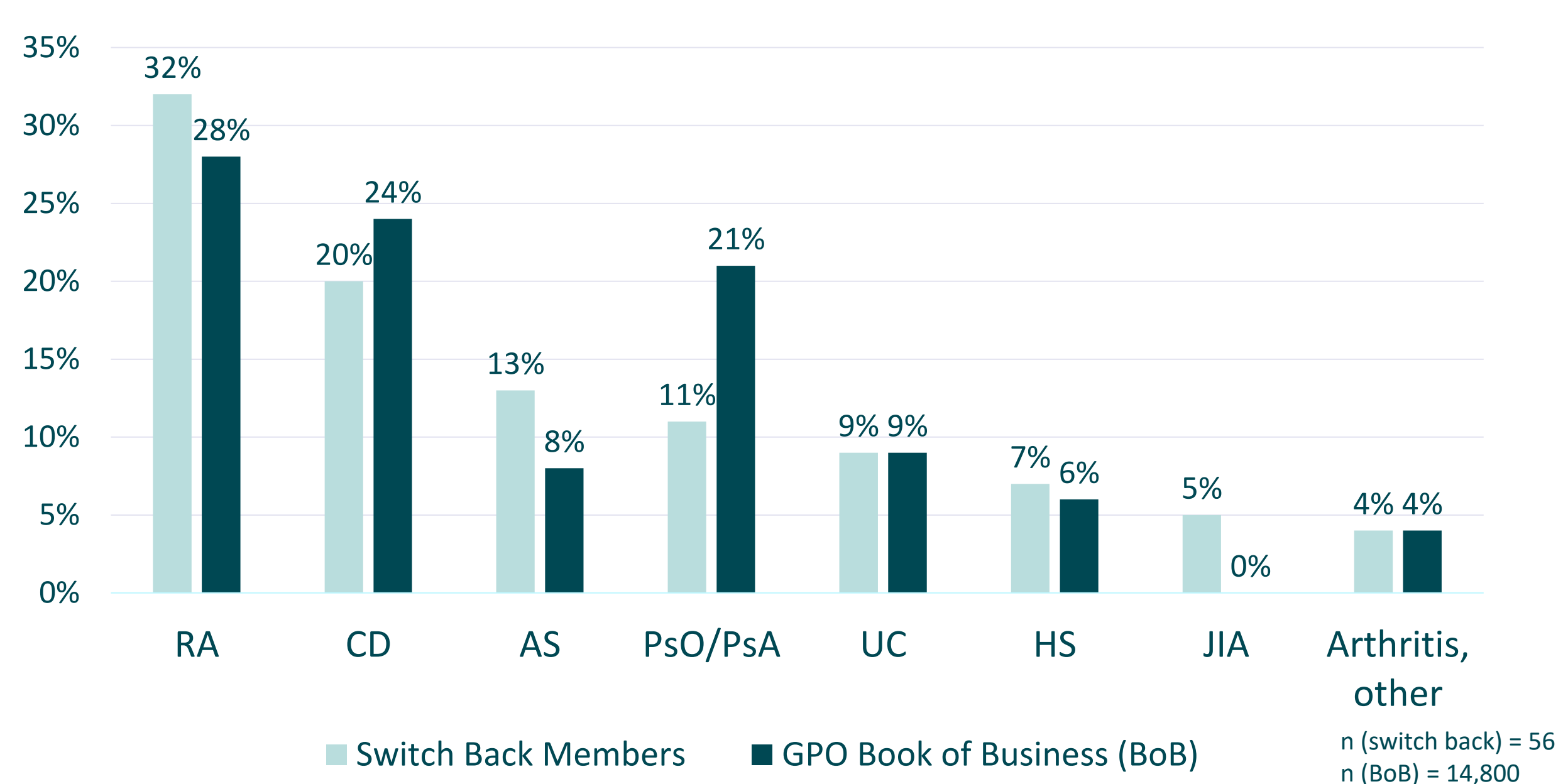
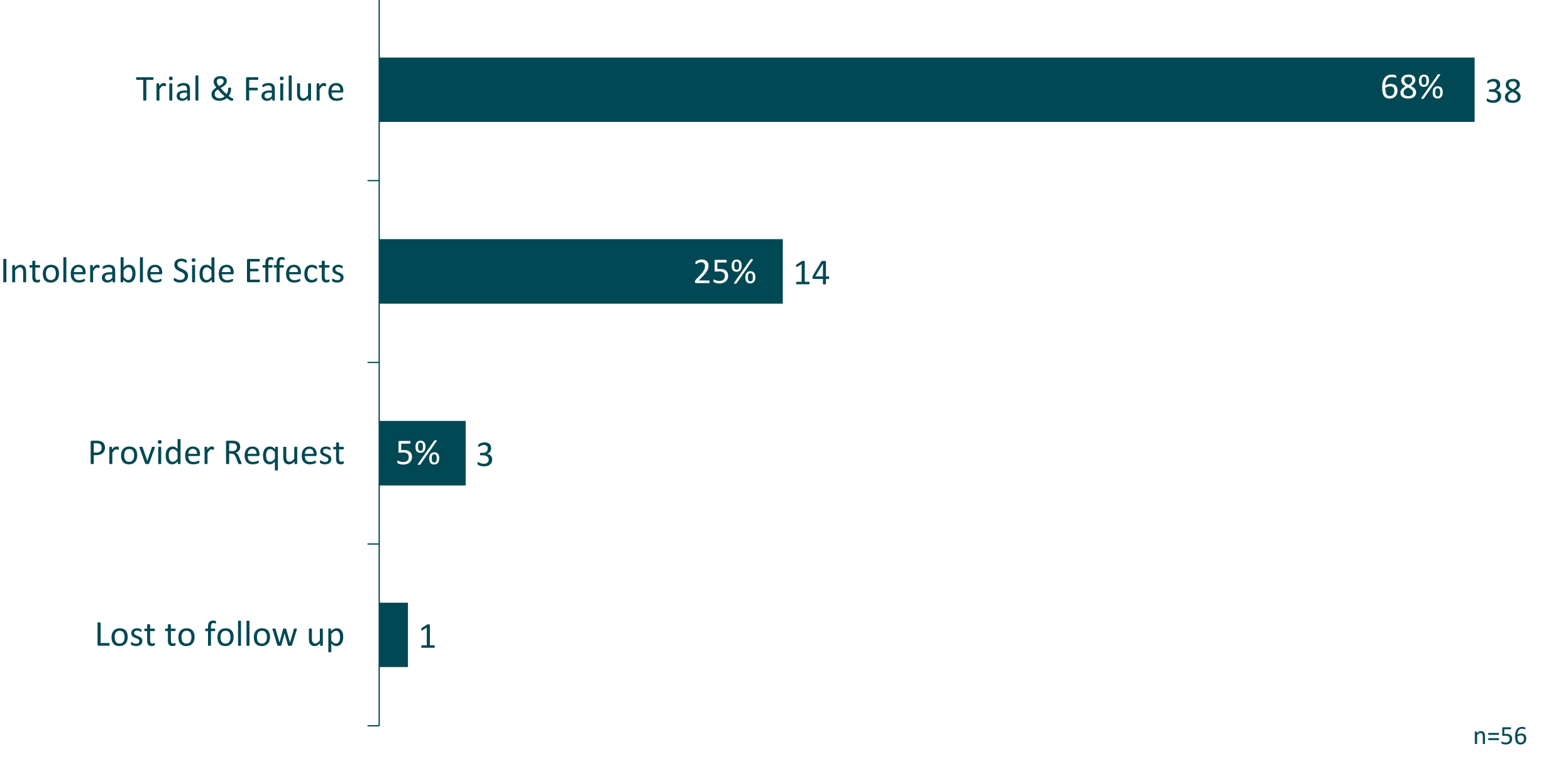


Figure 4: Switch Back Prior Authorization Reasoning



Discussion

- Almost 80% of branded adalimumab utilizers switched to the formulary-preferred adalimumab biosimilar after the branded adalimumab was excluded in April 2024 (Table 1).
- After the formulary exclusion of branded adalimumab, 9% of utilizers “product-hopped” to a different branded biologic product instead of switching to an adalimumab biosimilar. (Table 1: Indirect Switch).
- The most prevalent biologic drug products that indirect switch members utilized were secukinumab (26%), upadacitinib (20%), and risankizumab (16%) (Figure 1).
- The most common disease states among indirect switch members were PsO/PsA (36%), RA (26%), and HS (15%) (Figure 2).
- The choice of alternative biologic drug product was driven by member diagnosis with majority of RA members switching to etanercept (38%), PsO/PsA and CD members switching to risankizumab (35%), and HS members switching to secukinumab (100%) (Table 3).
- Of the switch back members, the majority were diagnosed with RA (32%) followed by CD (20%) and AS (13%) (Figure 3). This may raise effectiveness concerns with adalimumab biosimilars in comparison to branded adalimumab in the treatment of these conditions.
- When compared to the Employers Health BoB prescribing patterns for branded adalimumab, switch back members generally followed the same prevalence of adalimumab indications, with the exception of PsO/PsA and CD (Figure 3). This discrepancy may indicate that members with PsO/PsA or CD have less of a perceived need to switch back to the branded product from the biosimilar.
- When reviewing the prior authorizations for switch back members, the most common reasoning provided by prescribers was the trial and failure of the biosimilar product (68%).
- Provider requests for members to switch back to branded adalimumab without any other reasoning was 5%. This indicates that provider hesitancy towards biosimilar prescribing may not be as prevalent as previously expected but is still a factor (Figure 4).

Conclusion and Future Outlook

- While a majority of the members impacted by the formulary exclusion of branded adalimumab switched to the formulary preferred biosimilar, a notable proportion either switched to a different branded biologic or returned to branded adalimumab.
- The disease state of the member may influence the decision to “product-hop” as well as the choice of which product to switch to.
- Members who unsuccessfully switch to a biosimilar are more likely to try a different branded biologic product than go back to branded adalimumab.
- Unsuccessful switches to the formulary preferred biosimilar are mostly due to trial and failure or intolerable side effects, not prescriber preference.
- Further research is needed to analyze financial savings of switching to a biosimilar product and to continue determining, on a larger scale, why patients may not switch to a biosimilar.